Situation Report

Nigeria Centre for Disease Control and Prevention

Protecting the health of Nigerians

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Ebola Disease Situation Report

5th November 2026

Key Points

In the last 24 hours

• One (1) additional contact completed the mandatory 21-day follow-up period today and exited the quarantine facility. Cumulatively, seven (7) contacts have successfully completed the 21-day follow-up period and exited the facility.

• No new contacts were identified today

• No symptomatic contacts were identified today

• Signals involving suspected cases were received from two (2) states today. Investigations, sample collection and transportation to designated testing laboratories are pending.

• Contact tracing and monitoring remain ongoing. A total of 62 contacts linked to the index case have been identified, with seven (7) having completed follow-up and exited the facility.

• Exposure remains confined to a single epidemiological cluster directly linked to the index case.

• The National Incident Management System (IMS remains activated at the highest level, providing strategic leadership and coordination of the ongoing response.

Table 1: Summary table of contacts as of 5 November 2026

Cumulative (new in the last 24 hours)		
62 (0) **Contacts listed** **61 local, 1 evacuated**	**7 (+1)** Contacts exited quarantine facility	**0** **Contacts tested positive**

Epidemiological summary of the confirmed case

A 42-year-old woman with a travel history from Uganda via Addis Ababa arrived in Abuja on 3 October 2026. Symptoms commenced on 7 October, and she was admitted to a private health facility on 11 October. NCDC was notified on 19 October, prompting case investigation and specimen collection. The National Reference Laboratory confirmed Ebola virus disease on 20 October.

Response activities by pillar

Pillar	Key activities to date	Next steps
Coordination	• National IMS activated with high level coordination by the Coordinating Minister of Health & Social Welfare • High level stakeholder meeting held with partner agencies to strengthen coordination, resource mobilization and response accountability.	• Continue coordination across one health sectors and partners • IAP Implementation has commenced
Surveillance and contact tracing	• Two (2) new signals turned suspected cases • Sixty -Two (62) contacts identified • Contacts are expected to complete follow-up by November • Seven (7) contacts successfully completed and exited their mandatory 21-day follow-up period • Expected completion dates range from 31 Oct to 13 Nov 2026. • Contact tracing and monitoring ongoing. • Review and expansion of scope and strategy of contact identification concluded	• Review the Guidelines on Contact tracing, Self Quarantine, Selfcare kit, and Ebola Call protocol • Develop EVD dashboard on SORMAS • Create dashboard for contact management • Engage and assess additional contacts for possible evacuation to the quarantine facility • Continue follow-up of contacts in quarantine • Continue contact tracing and monitoring • Disseminate reviewed case definitions to all stakeholders
Laboratory	• Laboratory confirmation reported on 20th October 2026 • Phylogenetic sequencing concluded by NCDC NRL & The African Centre of Excellence for Genomics of Infectious Diseases (ACEGID) • Validated SOPs and job aids on sample reception, molecular detection, and genomics sequencing. • So far 27 samples have been tested, out of which 1 is positive (index case) while 24 are negative • The 26 negative samples include 22 samples from the contacts of the index case and 4 from suspected cases	• Awaiting the samples from 2 states • Expand testing capacity across the laboratory network prioritizing states with international Ports of Entry (PoE) to strengthen early detection and response • Maintain uninterrupted EVD testing and confirmation at the NRL, with expansion to Lagos • Continue diagnostic testing and maintain laboratory records. • Continue diagnostic testing and maintain laboratory records • Proposition additional laboratory reagents and commodities for the national network for laboratories for EVD testing • Disseminate standardized technical SOPs and guidelines for specimen management and molecular detection to promote consistency and quality in laboratory operations

Pillar	Key activities to date	Next steps
Case management	• Commodities approved for AUTH • Commenced follow-up engagement with contacts that exited mandatory quarantine period • Conducted a virtual Mental Health and Psychosocial Support (MHPSS) town hall meeting for contacts who had been under quarantine. • Conducted a total of ten (10) individualized MHPSS counselling sessions for distressed contacts • Held a meeting with the FCT Case Management Team to discuss case management activities • Continue to assess and treat contacts who developed symptoms that may be unrelated to EVD • Developed discharge criteria and follow-up • Engaged with FCT team and the contact tracing pillar for the clinical care and psychosocial support for contacts in quarantine facility • Secured a dedicated ambulance and placed on standby for safe and rapid patient referral	• Planning for the next webinar- Management of EVD contacts, specifically effective communication, emotional intelligence and psychosocial support • Review and disseminate approved procedures & guidelines for triage, isolation, supportive care (including psychosocial support), and discharge • Sensitisation of HCWs on high index • Establish referral mechanisms for suspected cases from point of detection to treatment centres • Identification and optimisation of treatment centres in states with designated POEs
Infection prevention and control (IPC)	• Trained 45 FCT Healthcare workers in the 2nd batch of IPC capacity building • Conducted IPC training for 18 housekeepers at the facility where the contacts are housed • Held bilateral meetings between with the CMD of UATH on case management capacity and support • Conducted Inspection and Assessment of Isolation centre • Supervised safe medical evacuation of the index case ensuring protection of health care workers • Engagement of current managing facility on IPC measures including guidelines and SOPs for standard and transmission-based precaution	• Continue to implement Healthcare worker protection, investigation and monitoring. • Training of Isolation Staff scheduled • Refresher training for EHOs and terminal cleaning at the house of the index case for medical evaluation • Implement the IPC ring strategy across concentric perimeters to strengthen screening and isolation measures • Conduct IPC assessment for surge capacity for isolation in Karu General Hospital

Pillar	Key activities to date	Next steps
Risk communication and Community Engagement (RCCE)	• Conducted sentiment analysis to gauge public emotional dynamics on Ebola for enhanced messaging and targeted public engagement • Engaging social media influencers for amplification of Ebola key messages and rumour countering and management. • Commenced deployment of infodemic rumour log tools to aid tracking and countering misleading information. • The RCCE pillar provided feedback and responses to online sentiments expressing political distrust, perceived politicisation of the Ebola outbreak, and conspiracy narratives questioning government motives • Disseminated the social listening insights report to relevant stakeholders and partners to inform response actions • Provided public advisory on Ebola Disease preventive measures • Conducted social listening on Ebola Disease and generated insight for infodemic management	• Continue to actively monitor and counter misinformation through online and offline social listening • Orientation. trainings and engagement forums for schools' management and religious leaders on Ebola awareness • Develop guidance on EVD for schools and mass gathering • Engage faith based-organisations, Media-EIS Fellows and Social Media Influencers • Sustain regular and proactive two-way communication across all platforms
Logistics	• Quantification of consumables for University of Abuja Teaching Hospital • Supplies prepositioned for practical drills for the EVD Training of Trainers (TOT) for IPC focal persons and frontline HCWs • Received donated items from Africa CDC, DCL Laboratories and Nigeria Red Cross Society. • Continue to consolidate list of requirements from the designated treatment facilities and response teams from specific pillars • Provided field Support for Surveillance Teams & EVD Care Kits for Quarantined contacts • Enhanced PPEs and other essential supply stockpile • 500 donated laboratory testing kits added to stockpile	• Release and dispatch priority commodities to the FCT against validated needs **immediately** • Coordinate mechanism to maintain adequate levels of supplies • To provide and preposition additional PPEs and essential supplies to high-risk states
POE	• In-depth investigation led by the Port Health Services with other aviation stakeholders to support response • Heighten surveillance are being carried out at all the 5 international airports • Sensitization and trainings are ongoing • Donation of PPEs and IPC materials were received from various partners	• Continue enhanced surveillance at all ports of entry

Data source

Information for this disease is case-based data retrieved from the case investigation and laboratory reports.

Key definitions

Case Definitions for Ebola Disease Version 3 (22nd October 2026)

Suspected case:

Any person with sudden onset of fever and at least three of the following symptoms: - headaches, lethargy, anorexia / loss of appetite, aching muscles or joints, stomach pain, difficulty swallowing, vomiting, difficulty breathing, diarrhoea or hiccups in Nigeria WITH Epidemiological or travel linkage to Democratic Republic of

Congo (DRC), Kenya, Uganda or other countries reporting suspected or confirmed case(s) of Ebola in the proceeding 21 days

AND/OR

Any person with unexplained bleeding in Nigeria WITH OR WITHOUT travel history to any of DRC, Kenya, Uganda, other countries reporting suspected or confirmed case(s) of Ebola in the proceeding 21 days

AND/OR

Any person with sudden onset of fever with or without any of the following symptoms; headaches, lethargy, anorexia / loss of appetite, aching muscles or joints, stomach pain, difficulty swallowing, vomiting, difficulty breathing, diarrhoea or hiccups and having had contact with a confirmed or probable case of Ebola within the proceeding 21 days

AND /OR

Any person with sudden onset of fever with or without any of the following symptoms; headaches, lethargy, anorexia / loss of appetite, aching muscles or joints, stomach pain, difficulty swallowing, vomiting, difficulty breathing, diarrhoea or hiccups and who is not responding well to treatment of any other cause of fever and have had contact with wildlife.

Probable case:

Any suspected case deceased or alive, where it has not been possible to collect specimens for laboratory confirmation having an epidemiological link with a confirmed case.

Laboratory confirmed case:

Any suspected or probable case with a positive laboratory result.

Laboratory confirmed case(s) must test positive for the virus antigen, either by detection of virus RNA by reverse transcriptase-polymerase chain reaction (RT- PCR), or by detection of IgM antibodies directed against Ebola.

COMMUNITY BASED SURVEILLANCE CASE DEFINITION:

Any person with onset of fever and no response to treatment of usual causes of fever in the area;

OR

Any person with at least one of the following signs: bleeding, bloody diarrhoea, bloody urine;

OR

Any sudden or unexplained death

Note: If an alert case (living or dead) is identified, report the case to a surveillance team or to the nearest health facility

Contact:

Any person who has had direct or indirect exposure to the blood, body fluids, or tissues of a person or animal infected with Ebola virus, particularly when the infected person is symptomatic.

Public Messages

Suspected Ebola needs professional assessment and appropriate care, not an improvised home-treatment arrangement. **Call 6232** for help and keep children and unnecessary visits away from close contact with sick person(s).

- Ebola spreads through direct contact with blood or body fluids of a sick or deceased person, or contaminated materials
- A person cannot spread it before symptoms begin to appear. [a,b]
- Stay calm and vigilant; no need for panic.
- Practice hand hygiene and avoid body fluids of suspected cases.
- Watch for fever, severe weakness, headache, vomiting and diarrhoea, especially after possible exposure.
- Report any exposure immediately, even if well. If symptomatic, avoid close contact and call public health authorities before going to a facility.
- Cooperate with contact tracers, including relevant airline passengers.
- Avoid rumours and stigma; use only official sources.

QUICK REFERENCE

Nigeria Centre for Disease Control and Prevention: www.ncdc.gov.ng

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